

Urine - Blood In - Female (Child)

Algorithm 1480 | Child (0-17 years) | Gender at birth: Female

Definition

- UAT-only adult and pediatric assessment of reported visible blood in urine, separating life-threatening bleeding, retention/obstruction, infection, trauma and pregnancy-related concerns from stable haematuria that still requires in-person testing.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] Is blood definitely mixed with urine, and is it pink, red or dark brown, streaked, clotted, or causing heavy bleeding? Could it instead be vaginal/menstrual, rectal, skin or genital bleeding?
2. [DURATION] When was it first and last seen, how many times, and is it continuing or increasing?
3. [YES_NO] Is there inability to urinate, only drops despite a full painful bladder, clots, severe side/back/abdominal/groin pain, fever or rigors, vomiting, reduced urine, swelling, confusion, faintness, pallor, or serious illness?
4. [YES_NO] Was there recent abdominal, back, pelvic or genital trauma, a procedure or catheter problem, strenuous exercise, or possible foreign body?
5. [OPEN_TEXT] What is the patient's age and anatomy, and are they pregnant or recently postpartum, taking anticoagulant/antiplatelet medicines, or known to have kidney, urinary, bleeding, immune or cancer conditions?
6. [YES_NO] For a child or adolescent, is there fever, abdominal mass/swelling, bruising, genital injury/discharge, reduced urine, marked distress, or any privacy or safeguarding concern?
7. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Flank Pain

Is pain in the side or back the main concern rather than visible blood in urine?

Pain focused in the side or back belongs in the Flank Pain pathway.

1. Disposition level 100

Is there heavy or ongoing bleeding, clots with inability to urinate or painful bladder distension, fainting/shock, severe illness or confusion, severe trauma, severe flank/abdominal pain with vomiting or reduced urine, or an unwell infant or child?

Heavy bleeding, clot retention, shock, serious infection, obstruction and significant urinary-tract trauma require emergency assessment. A child may deteriorate without being able to describe symptoms reliably.

1. Disposition level 70

With no emergency feature, is there any possible visible blood in urine, including a first, single, painless or small episode, or blood with dysuria, fever, flank pain, pregnancy/postpartum status, anticoagulant use, recent trauma, or childhood?

NHS.UK advises prompt assessment of any visible haematuria. Urine testing and sometimes blood tests, examination or imaging are needed; food, medicines or menstruation must not be assumed to explain it remotely. Unexplained visible haematuria in a child warrants very urgent specialist assessment under NICE suspected-cancer guidance.

Advice

Qatar urinary-bleeding emergency response

- Call Qatar emergency services on 999. Do not allow self-driving. Do not force fluids when urine cannot pass, the bladder is painfully full, vomiting is ongoing, or emergency assessment is required. Do not insert, flush or manipulate a catheter unless specifically trained and directed. Report prescribed anticoagulant/antiplatelet medicines, but do not stop them unless the emergency clinician directs it. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: increasing bleeding or clots; cannot urinate; fainting or confusion; worsening pain; reduced responsiveness.

Prompt in-person haematuria assessment

- Arrange prompt in-person assessment through an approved Qatar adult or pediatric pathway for urine testing and clinician-directed examination, blood tests or imaging. Do not assume food, menstruation or medicine is the cause and do not start leftover antibiotics. Children with unexplained visible blood need a very urgent age-appropriate pathway. Call 999 for heavy bleeding, clots with retention, severe pain/illness, fainting, confusion or deterioration. The exact Qatar non-emergency urology, nephrology, pediatric, maternity and trauma destinations are local approval required.
- Call Back If: heavy bleeding or clots; cannot urinate; fever or rigors; flank pain or vomiting; reduced urine; child becomes unwell.

First aid

- Call Qatar emergency services on 999. Do not allow self-driving. Do not force fluids when urine cannot pass, the bladder is painfully full, vomiting is ongoing, or emergency assessment is required. Do not insert, flush or manipulate a catheter unless specifically trained and directed. Report prescribed anticoagulant/antiplatelet medicines, but do not stop them unless the emergency clinician directs it. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NHS.UK, "Blood in urine", <https://www.nhs.uk/symptoms/blood-in-urine/> (accessed 2026-07-25)

- NICE NG12, "Suspected cancer: recognition and referral", recommendations for visible haematuria in adults and children (updated 2026)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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